

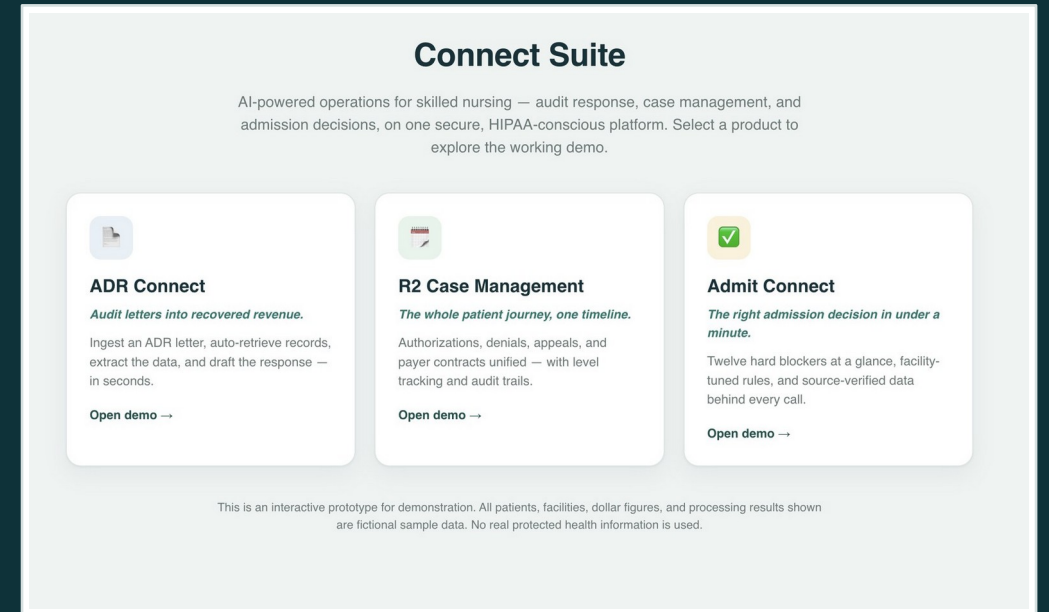
## CONNECT SUITE

# Audit response, case management, admission decisions.

AI-powered operations for skilled nursing. One platform, a nurse in the loop, and a source document behind every answer.

Prepared by Insight Ally

Justin Polomcean · [jpolomcean1228.github.io/adr-suite](https://jpolomcean1228.github.io/adr-suite)



Working prototype — all patients, facilities and figures are fictional sample data.

# Audit volume is rising. The clock is not.

*Every ADR is a fixed-deadline, full-episode document pull — handled by clinical staff who have other jobs.*

## 45

days to respond

The window is fixed from the date of the letter. Miss it and the claim denies automatically, with no clinical review at all.

## 1 ADR

= the entire episode

For SNF claims one request covers every record from admission through final discharge, not a single date of service.

## 5-claim

probe and educate

CMS opened routine review of every nursing home in 2023. RAC complex reviews sit on top of that.

**The facility does not control how many letters arrive.  
It controls only how fast and how completely it answers them.**

# Three leaks, all of them staffed by nurses

*Each one is a manual search across systems, under deadline, by someone whose licence is needed on the floor.*

1

## Audit response

Records pulled by hand from PointClickCare and NetHealth, assembled into a package, and submitted before the deadline.

*Hours of clinical time per letter*

2

## Denials and appeals

An incomplete or late package denies a claim the clinical care already earned. The appeal then costs more than the response would have.

*Recoverable revenue written off*

3

## Admission decisions

A referral arrives with a payer, a level and a diagnosis. Someone decides — often without the contract terms in front of them.

*Empty beds, or the wrong admission*

# Every open letter, every deadline, every dollar

The worklist ranks by what is at risk and when it is due — so the response order is obvious before anyone opens a chart.

DOLLARS AT RISK

\$34,100

RECOVERED YTD

\$1.24M

OPEN ADRS

2

OVERDUE

1

ADR Worklist

1 queued for processing

| RESIDENT          | FACILITY       | PAYER            | AT RISK  | DEADLINE    | STATUS    |         |
|-------------------|----------------|------------------|----------|-------------|-----------|---------|
| Eleanor Whitfield | Folksbrook SNF | Medicare Part A  | \$18,450 | in 3 days   | Submitted | —       |
| Marcus Delgado    | Spring Creek   | Humana MA        | \$9,720  | in 6 days   | Queued    | Process |
| Ruth Ann Boyle    | Amherst Center | Medicare Part A  | \$24,380 | in 1 day    | Due Soon  | Process |
| James P. Okafor   | Two Rivers     | UnitedHealthcare | \$12,110 | 12 days ago | Submitted | —       |
| Gloria Nakamura   | Journey Health | Aetna MA         | \$15,600 | in 8 days   | Submitted | —       |

Click **Process** on a queued letter to watch the automated pipeline retrieve records, extract data with multi-agent arbitration, and draft a response.

\$34,100

at risk right now

\$1.24M

recovered year to date

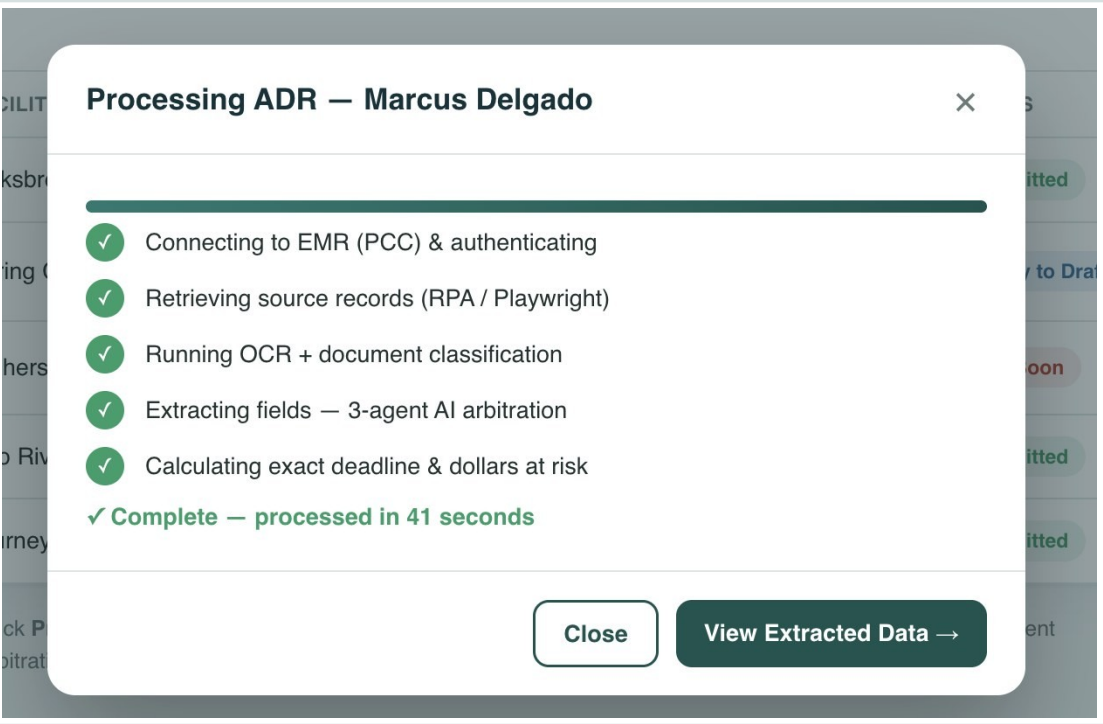
1

overdue — visible, not buried

Medicare Part A, Humana MA, Aetna MA, UnitedHealthcare — one queue.

# Forty-one seconds, end to end

Connect to the EMR, retrieve the records, classify them, extract the fields, and calculate the deadline and the dollars at risk.



Prototype with fictional sample data.

- 1

**Connect and authenticate**  
Into PointClickCare, with facility credentials.
- 2

**Retrieve source records**  
RPA and Playwright pull the full episode.
- 3

**OCR and classify**  
Every document typed and ordered.
- 4

**Extract with arbitration**  
Three agents read, then must agree.
- 5

**Calculate exact exposure**  
Deadline and dollars at risk, deterministically.

The nurse's job starts after this runs, not before it.

# Every field carries a confidence score and a source

Three models read the same record and arbitrate. Where they disagree the score drops and the reviewer is sent straight to that field.

Extracted Data — Marcus Delgado

RESIDENT

Marcus Delgado 99%

DATE OF SERVICE

03/14/2025 – 04/02/2025 99%

LETTER DATE

05/06/2025 99%

SUBMISSION DEADLINE

06/05/2025 (exact) 99%

PAYER / CONTRACTOR

Humana MA · Noridian 92%

TOTAL CHARGES → AT RISK

\$9,720 99%

LEVEL OF CARE

Skilled — Level 3 92%

DOCUMENTS RETRIEVED

14 of 14 99%

Each value is cross-checked by three AI models with arbitration; confidence scores shown. [View source records](#)

Close

Generate Response Letter →

Prototype with fictional sample data.

## Cross-checked, not averaged

Each value is checked by three AI models with arbitration. Agreement is earned, not assumed.

## Confidence made visible

99% on the dates and the dollar figure. 92% on payer and level of care — exactly where a human should look first.

## 14 of 14 documents retrieved

The count is shown because a missing document is the most common reason a response fails.

## Source records one click away

Missing or ambiguous data is reported, never invented.

# The response arrives written

*Drafted in the analyst's standard voice from the extracted data, addressed to the right contractor, and editable before anything is sent.*

Draft Response — Marcus Delgado

Noridian Healthcare Solutions — Medical Review

Re: Additional Documentation Request — Marcus Delgado

To Whom It May Concern:

Enclosed please find the complete medical record supporting skilled services rendered to the above resident from March 14 through April 2, 2025. The documentation demonstrates daily skilled nursing and therapy at a Level 3 acuity, including physician certification, MDS assessments, and the therapy minutes required to substantiate the billed period.

We respectfully request full reimbursement of the \$9,720 at issue and remain available to provide any further documentation needed to resolve this review.

Sincerely,  
ADR Prep Team, on behalf of Spring Creek

Drafted in the analyst's standard voice from extracted data. Editable before sending.

Edit

Send via Secure Portal

- Addressed to the contractor

Noridian Healthcare Solutions, Medical Review — the right recipient, the right reference, the right resident.
- The dollars are stated

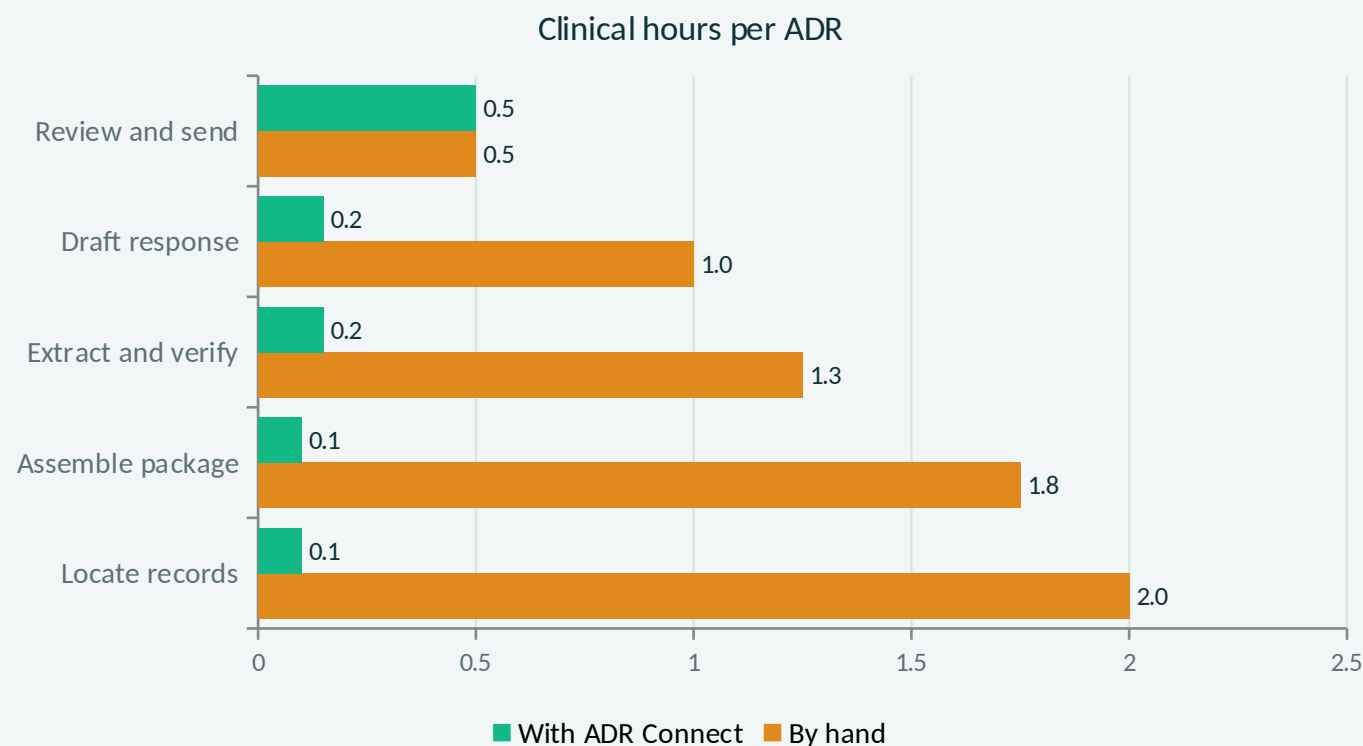
Full reimbursement of the \$9,720 at issue, taken from the extracted figure rather than retyped.
- Written in your voice

The analyst's standard letter structure, not a generic template that reads like software wrote it.
- Editable before sending

Send via secure portal — the submission path CMS already expects.

# The same letter, two workflows

*Illustrative time model — replace with Zimmet's measured baseline before this is shown to a client.*



6.5 → 1.0

hours of clinical time per audit letter

The half hour that remains is the half hour that matters — a clinician reading what the system produced and deciding whether it is right.

*Retrieval time does not scale with episode length. A 90-day stay costs the same as a 20-day stay.*

# Who is open, at what level, with how much outstanding

Residents, facility, primary payer, current level and open item count — the working list a case manager actually needs.

R2 Case Management

PROTOTYPE · SAMPLE DATA

| Residents      |                |                        |               |            |        |
|----------------|----------------|------------------------|---------------|------------|--------|
| RESIDENT       | FACILITY       | PRIMARY PAYER          | CURRENT LEVEL | OPEN ITEMS |        |
| Harold Simmons | Spring Creek   | Humana MA (In-Network) | L2→L3         | 2          | Open → |
| Beatrice Lowe  | Amherst Center | Medicare Part A        | Level 3       | 1          | Open → |

Level changes are tracked as state

L2→L3 is not a note in a chart. It is a status with an owner, a date and a payer response outstanding.

Payer visible at the row level

Humana MA in-network and Medicare Part A behave differently. The difference is on screen before anyone opens the case.

Open items counted, not remembered

Two for one resident, one for another. Nothing depends on whether someone remembered to follow up.

# A level-up is worth \$132 a day

The timeline and the contract sit side by side, so the gap between the level authorized and the level delivered is visible while it can still be closed.

HAROLD SIMMONS — SPRING CREEK

Patient Timeline

Apr 2, 2025

Admission

Skilled admit from Mercy General

Apr 3, 2025

Authorization Requested

Level 2 · 7 days

Apr 10, 2025

Authorization Approved

Level 2 · through Apr 17

Apr 15, 2025

Level-Up Review

Eligibility flagged

Today

Level-Up Requested

Level 2 → Level 3

Payer Contract

view source

|                  |                    |
|------------------|--------------------|
| Line of business | Medicare Advantage |
| Level 2 rate     | \$612 / day        |
| Level 3 rate     | \$744 / day        |
| Network status   | In-Network         |
| Auth cadence     | Every 7 days       |

Level Authorization

Current level

Level 3 (pending)

Level-up request submitted — awaiting payer response. Event added to timeline.

\$612 → \$744

Level 2 to Level 3 per diem, on this contract

A twenty-day stay authorized one level low is \$2,640 of care delivered and not billed.

The clock is on screen

Authorization approved through Apr 17, level-up requested today, payer response outstanding — with the 7-day auth cadence from the contract beside it.

The contract is attached

Line of business, both level rates, network status and cadence, with the source document one click away.

Nothing is reconstructed later

When the appeal comes, the sequence is already written down in the order an auditor wants to read it.

Prototype with fictional sample data. Rates shown are from the sample contract.

# Clear to admit — all twelve blockers pass

Twelve facility acceptance criteria checked against source-verified data — the bed is offered, or it is not.

Admit Connect

PROTOTYPE - SAMPLE DATA

Dorothy Kessler

Franklin Ruiz

REFERRAL

FACILITY

REFERRAL SOURCE

REASON

Dorothy Kessler, 78

Journey Health & Rehab

Mercy General (Discharge Planning)

Post-CVA, rehab

✓

Clear to admit — all 12 hard blockers pass this facility's criteria.

HARD BLOCKERS — FACILITY ACCEPTANCE CRITERIA

✓

Ventilator

Not required

✓

Bariatric limit

215 lbs - within limit

✓

Dialysis

Not required

✓

IV therapy

Facility capable

✓

Behavioral / secure unit

Not required

✓

Isolation / infection

No active precautions

✓

Bed availability

2 beds open

✓

Insurance / auth

Medicare A verified

✓

Wound care level

Stage 2 - within scope

✓

Medication admin

All on formulary

✓

Therapy availability

PT/OT/ST staffed

✓

Acuity vs staffing

Matches current ratio

✓ Admit

✕ Decline

© Decision-ready

## Medicare A verified at the door

Which part pays, and whether the authorization exists, resolved before the bed is offered rather than discovered at billing.

## Twelve criteria, one screen

Ventilator, bariatric limit, dialysis, IV therapy, behavioral unit, isolation, beds, insurance, wound care, medication, therapy, acuity versus staffing.

## Decision-ready

Under a minute from referral to a defensible answer — the empty bed filled the same day instead of the next one.

An empty Medicare A bed is roughly \$600 a day of unbilled capacity.

# And when the answer is no, it is no for stated reasons

Three hard blockers detected — ventilator, isolation and acuity versus staffing — each named, sourced, and reviewable before anyone commits.

Dorothy KesslerFranklin Ruiz

REFERRAL

Franklin Ruiz, 64

FACILITY

Journey Health & Rehab

REFERRAL SOURCE

St. Anne's (Case Mgmt)

REASON

Resp. failure, vent-dependent

3 hard blockers detected — Ventilator, Isolation / infection, Acuity vs staffing. Review before admitting.

HARD BLOCKERS — FACILITY ACCEPTANCE CRITERIA

Ventilator

Vent-dependent · no vent unit

Bariatric limit

190 lbs · within limit

Dialysis

Not required

IV therapy

Facility capable

Behavioral / secure unit

Not required

Isolation / infection

Active MRSA precautions

Bed availability

2 beds open

Insurance / auth

Humana MA verified

Wound care level

Within scope

Medication admin

On formulary

Therapy availability

PT/OT/ST staffed

Acuity vs staffing

Exceeds current night ratio

Admit

Decline

© Decision-ready

The wrong admission is expensive too

A resident the facility cannot safely staff costs more than the empty bed — in agency cover, in transfer, and in survey risk.

Reasons, not a score

Vent-dependent with no vent unit. Active MRSA precautions. Exceeds current night ratio. Each one is a fact a nurse can act on.

Same screen, both answers

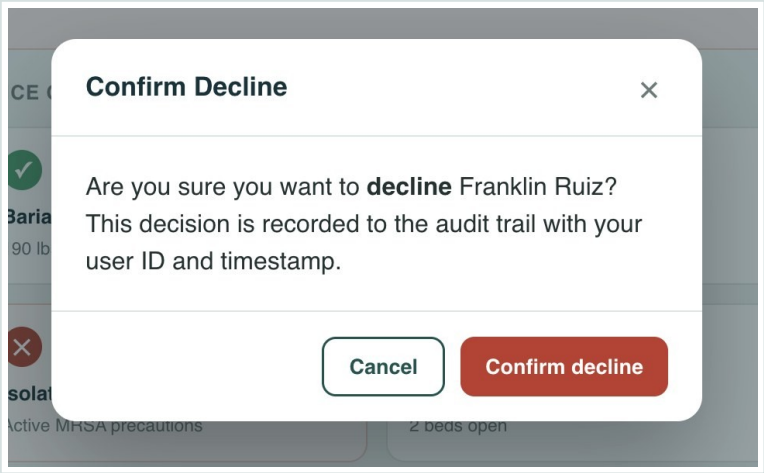
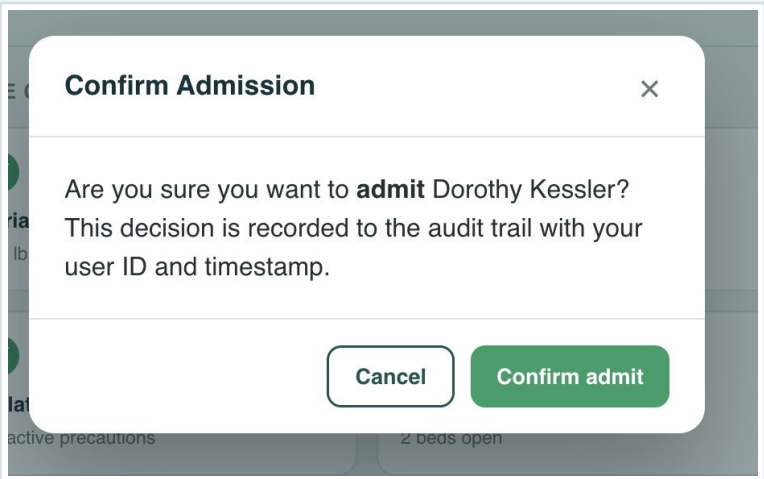
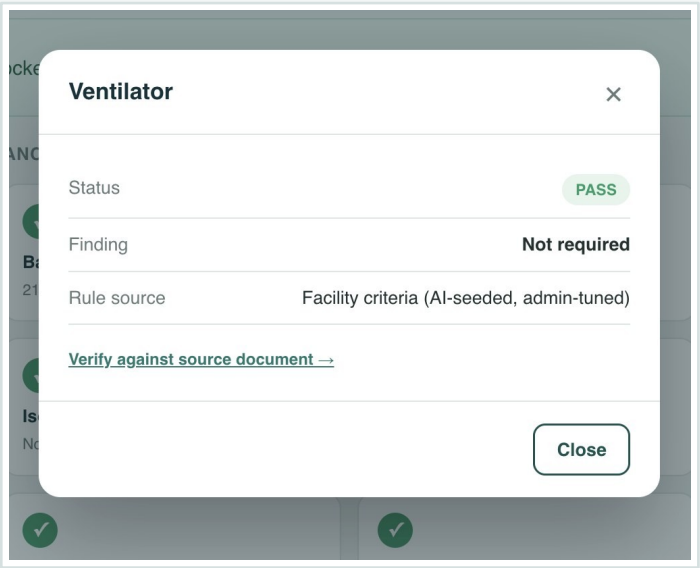
Admit and decline are the same workflow. The system does not push toward yes.

Declining fast is as valuable as admitting fast — it frees the referral source to place the patient somewhere appropriate.

Prototype with fictional sample data.

# Every call is checkable, and every call is recorded

Open any criterion to see its status, its finding, the rule that produced it, and a link to the source document behind it.



## Rule source is disclosed

Facility criteria, AI-seeded and admin-tuned. The facility owns its own acceptance rules; the system does not hide where a rule came from.

## Verify against source document

One link from the finding to the evidence. Trust is offered, not demanded.

## Recorded to the audit trail

Admit or decline, with the user ID and timestamp attached. When someone asks six months later why this patient was declined, the answer exists.

# A lower denial rate buys fewer future audits

*The part most vendors miss: under CMS rules your denial rate sets your own ADR limit.*

**CMS recalculates a provider's ADR limit from its denial rate. A clean review record moves the provider to a reduced cycle — a 0% denial rate can mean no reviews for three consecutive 45-day cycles.**

01

## Complete responses

Every requested element present, sourced and on time.



02

## Fewer denials

Claims stand because the record supports them.



03

## Lower denial rate

The measure CMS uses to set your review burden.



04

## Fewer ADRs next cycle

Less clinical time consumed by audit response.



05

## Capacity back on the floor

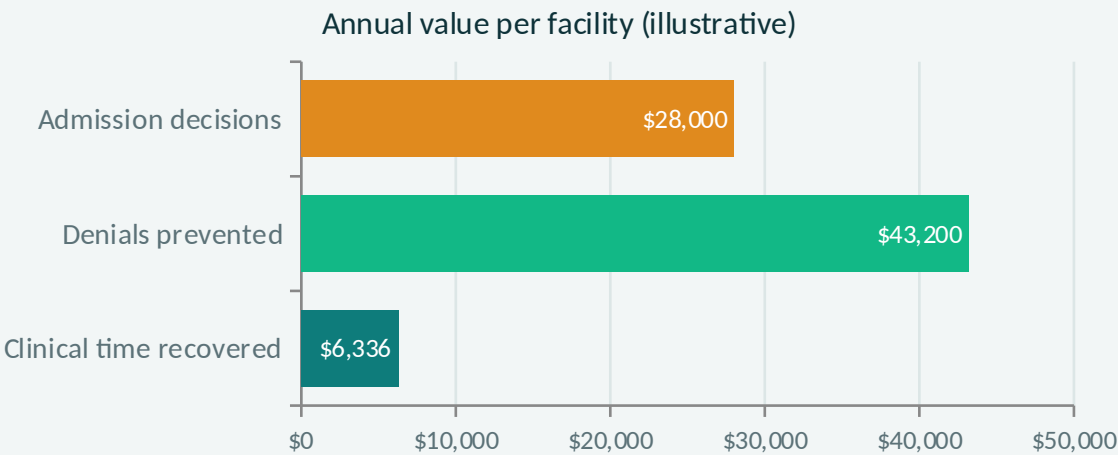
The saving compounds instead of plateauing.

# What one facility recovers in a year

Illustrative. Every input below is a placeholder to be replaced with Zimmet's actuals before this is shown to a client.

Assumptions

|                                 |           |
|---------------------------------|-----------|
| ADRs per facility per year      | 24        |
| Clinical hours per ADR, today   | 6.5       |
| Clinical hours with the suite   | 1.0       |
| Blended clinical labour cost    | \$48 / hr |
| Average episode value at risk   | \$18,000  |
| Denial rate, today → with suite | 22% → 12% |

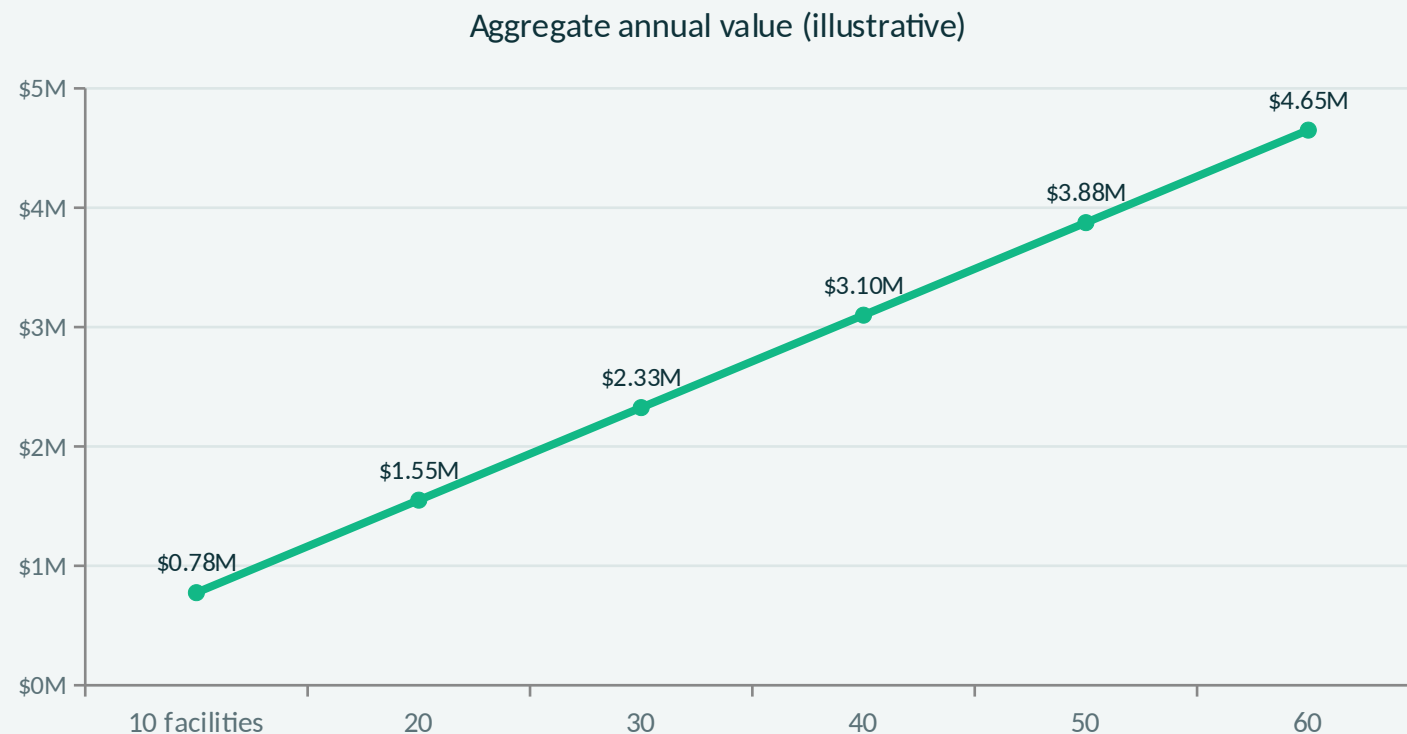


≈ \$77,500 per facility, per year

before counting the audits that never arrive

# The same model across a client base

*Illustrative, at \$77,500 per facility per year — linear because it is the same work in every building.*



Illustrative model for discussion. Facility count is a placeholder.

## Why it scales cleanly

The regulation is federal and the workflow is the same in every facility. Only the acceptance criteria differ, and those are configuration, not rebuild.

## Why Zimmet is the right partner

The reimbursement expertise already exists in the room. The suite is a delivery mechanism for advice Zimmet is already trusted to give.

## What it becomes

A managed service line rather than a software licence — priced against recovered revenue, not seats.

THE NEXT LAYER

# RegCheck Master

A nurse asks a question in plain language. The answer comes back grounded in the Code of Federal Regulations, with the citation attached.

*“Can I bill Part A for a resident who left for an outpatient procedure and came back the same day?”*

## Answered from the regulation

The response cites the governing section of 42 CFR, quotes the controlling language, and links the source — so the nurse can act and the MDS coordinator can verify.

757

rows of curated SNF Medicare compliance rules

200

question evaluation bank the answers are graded against

42 CFR

the governing authority, cited in every response

0

answers without a source — grounded, or it does not answer

# A compliance brain, a fleet of agents, and a system of record

*Three layers, each doing what it is actually good at — and none asked to be trusted with something it cannot prove.*

1

## The grounded brain

*RAG over curated regulation*

Retrieval-augmented generation across a curated corpus of SNF Medicare rules. Nothing is answered from memory; every response is drawn from a source that can be opened.

2

## The reading fleet

*Small single-purpose agents*

Each agent does one narrow job and cross-checks the others. Where they disagree, the conflict is escalated to a human rather than resolved silently.

3

## The system of record

*Smartsheet as rule library and ledger*

The auditable store that holds the rules and does the deterministic math. Calculations that must be exactly right are not left to a language model.

The model reads and drafts. The ledger calculates and records. The clinician decides. Tested before release against a document built with deliberate traps across 20+ fields, with an answer key.

## WHAT WE ARE PROPOSING

# A measured pilot, not a platform purchase

### Phase 1

## Baseline

Measure what an ADR actually costs today in two or three facilities — hours, denial rate, elapsed days. No software yet.

### Phase 2

## Pilot

Run ADR Connect and Admit Connect against live volume in those facilities, with a clinician approving every output.

### Phase 3

## Prove

Compare against the baseline on the same measures. Publish the result internally, whatever it says.

### Phase 4

## Scale

Extend across the portfolio and open RegCheck Master to floor staff as a service line.

*The baseline comes first because the claim has to survive being checked. If the numbers do not hold in phase 3, we say so.*